

YOUR FERTILITY JOURNEY GUIDE

Egg Freezing & Fertility Preservation
for Women of Color, Ages 25–38

Factual information • Body care tips • Provider guidance • Actionable checklist

1 in 6

couples affected by infertility
nationwide

2x

Black women more likely to experience
infertility than white women

50%

less likely for Black women to
seek fertility treatment

Sources: CDC National Survey of Family Growth • Shady Grove Fertility • Journal of Clinical Medicine (2024) American Society for Reproductive Medicine
• Fertility for Colored Girls • PubMed / NIH

Why This Guide Matters

Fertility decisions are deeply personal — and for women of color, they are also shaped by a healthcare landscape that has not always been equitable, inclusive, or well-informed about our unique medical needs. This guide was created to give you clear, research-backed information so you can make empowered choices about your reproductive future.

This guide is for informational purposes only and does not constitute medical advice. Always consult a board-certified reproductive endocrinologist (REI) before beginning any fertility treatment or egg-freezing cycle.

Who Should Use This Guide

- Women of color between the ages of 25–38 considering egg freezing
- Those who want to understand their fertility health before committing to a procedure
- Anyone navigating the fertility care system and wanting culturally informed guidance
- Women experiencing infertility who need trusted resources and community support

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Section 1

Understanding Fertility & Egg Freezing

What Is Egg Freezing?

Egg freezing — medically called oocyte cryopreservation — is a method of fertility preservation in which a woman's eggs (oocytes) are extracted, frozen, and stored for future use. When the woman is ready to attempt pregnancy, the eggs are thawed, fertilized with sperm via intracytoplasmic sperm injection (ICSI), and the resulting embryos are transferred to the uterus.

Egg freezing was considered experimental until 2012, when the American Society for Reproductive Medicine (ASRM) officially recognized it as a proven, non-experimental procedure. Since then, advances in vitrification (flash-freezing technology) have dramatically improved egg survival rates.

Why Consider Egg Freezing in Your 20s and 30s?

Egg quality and quantity naturally decline with age. The peak window for egg freezing is generally considered to be ages 25–35, when eggs are typically more plentiful and healthier. Freezing eggs earlier gives you more options later — whether due to career goals, relationship timing, medical conditions, or personal readiness.

Age at Freezing	Estimated Success Rate per Frozen Egg	Key Consideration
25–30	~5–7% per egg	Highest egg quality; ideal window
30–34	~4–6% per egg	Strong outcomes; still prime time
35–37	~3–5% per egg	Declining slightly; act proactively
38+	~2–4% per egg	Lower reserve; consult REI promptly

**Estimates based on IVF cycle data from SART (Society for Assisted Reproductive Technology). Individual results vary significantly based on health, clinic, and other factors.*

Common Reasons Women Choose to Freeze Their Eggs

- Career or educational goals — wanting to establish professionally before starting a family
- Not having the right partner yet — keeping biological options open
- Medical conditions — PCOS, endometriosis, premature ovarian insufficiency, or upcoming cancer treatment
- Family history of early menopause
- Desire to reduce age-related fertility risk while delaying parenthood
- Preserving fertility before gender-affirming treatments (for transgender individuals)

Important Note: Frozen eggs do not guarantee a future pregnancy. Success depends on many factors, including the number and quality of eggs frozen, your age at retrieval, and your overall health at the time of transfer. On average, experts recommend freezing 10–20 mature eggs to maximize the chance of at least one successful pregnancy later.

Section 2

Racial Disparities in Fertility Care

Understanding the systemic barriers that exist in fertility care is important for women of color entering this space. The disparities are real and well-documented — knowing them helps you advocate for yourself more effectively.

2x	~50%	8%
More likely: Black women experience infertility compared to white women	Less likely: Black women seek fertility treatment compared to white women	Black women who seek medical help to conceive vs. 15% of white women

Sources: Shady Grove Fertility (2024), The Prelude Network, Journal of Clinical Medicine

Key Barriers Women of Color Face

- **Medical mistrust:** A justified distrust of the healthcare system rooted in a history of exploitation and abuse, including non-consensual medical experiments on Black women.
- **Financial barriers:** Egg freezing typically costs \$10,000–\$15,000 per cycle plus \$500–\$1,000/year in storage fees. As of 2024, only 16 states mandate insurance coverage for fertility preservation.
- **Implicit bias:** Research shows implicit provider bias affects referral patterns and the quality of care given to women of color, sometimes leading to delayed diagnosis and treatment.
- **Cultural stigma:** In many communities of color, infertility is not openly discussed, creating shame and isolation. Myths about Black women being 'naturally fertile' persist and discourage help-seeking.
- **Lack of representation:** Few fertility specialists identify as Black or Hispanic, making it harder to find culturally concordant care.
- **Higher rates of specific conditions:** Black women are disproportionately affected by uterine fibroids and blocked fallopian tubes, which can complicate fertility and go undiagnosed longer.

How to Advocate for Yourself in Clinical Settings

You deserve thorough, respectful, and personalized care. If you feel dismissed, minimized, or rushed — you have every right to ask more questions, seek a second opinion, or switch providers. Your concerns are valid. Your symptoms are real.

- Bring a written list of symptoms and questions to every appointment
- Ask your provider to explain every test, result, and recommendation in plain language
- Request interpreters or translated materials if English is not your first language
- Look for clinics that explicitly state commitment to DEI and culturally competent care
- Seek out a fertility doula or patient advocate if navigating the system feels overwhelming

Section 3

The Egg Freezing Process, Step by Step

The full egg freezing process — from your first consultation to retrieval — typically takes 6 to 10 weeks. The stimulation and retrieval phase itself takes about 2 weeks per cycle.

Step 1	Initial Consultation & Fertility Assessment (Weeks 1–2) Meet with a reproductive endocrinologist (REI). You will receive baseline blood tests — including AMH (anti-Mullerian hormone) to assess ovarian reserve — and a transvaginal ultrasound to count antral follicles. These results help predict how your ovaries will respond to stimulation medications.
Step 2	Ovarian Stimulation (Days 1–10 of Cycle) You will self-inject hormone medications (FSH, LH, or combined) daily for approximately 9–12 days to stimulate your ovaries to produce multiple eggs at once. You will return to the clinic every 2–3 days for monitoring ultrasounds and blood draws to track follicle growth and hormone levels.
Step 3	Trigger Shot (~36 Hours Before Retrieval) When your follicles reach the right size, you will take a 'trigger shot' — typically hCG or a GnRH agonist — to finalize egg maturation. Timing is precise; this shot is usually given exactly 36 hours before retrieval.
Step 4	Egg Retrieval (Outpatient Procedure, ~20–30 Minutes) Under light sedation (IV anesthesia), your doctor inserts a thin needle through the vaginal wall using ultrasound guidance to aspirate eggs from each follicle. You will be awake but comfortable within about an hour and can go home the same day.
Step 5	Egg Assessment & Vitrification An embryologist immediately evaluates each retrieved egg for maturity. Mature eggs (MII oocytes) are flash-frozen using vitrification — an ultra-rapid cooling process that plunges them to -196°C in liquid nitrogen, preventing damaging ice crystal formation.
Step 6	Storage Frozen eggs are stored in liquid nitrogen at your clinic or a secure storage facility. With proper cryopreservation, eggs can remain viable for 10 or more years. Annual storage fees typically range from \$500 to \$1,000 per year.
Step 7	When You're Ready — Thaw, Fertilize, Transfer When you decide to use your eggs, they are warmed, fertilized with sperm via ICSI, cultured into embryos, and transferred to your uterus. This is a separate IVF cycle with its own costs and timeline.

Side Effects to Expect: Bloating, mild cramping, mood swings, and breast tenderness during stimulation are common and normal. Ovarian hyperstimulation syndrome (OHSS) is a rare but serious complication — your clinic will monitor you closely to minimize this risk.

Section 4

Body Care, Nutrition & Lifestyle

Eggs take approximately 90 days to mature before ovulation. That means the 3 months before your retrieval cycle is a prime window to support your body. While no lifestyle change can guarantee outcomes, research suggests that nutrition, stress management, and overall metabolic health all influence reproductive function.

Nutrition: What to Eat & Prioritize

A Mediterranean-style eating pattern — rich in vegetables, fruits, whole grains, legumes, healthy fats, and lean proteins — has been associated with nearly 40% higher IVF success rates compared to diets high in processed foods and fried foods (FertilityIQ, 2024).

Food Group	Why It Matters	Examples
Antioxidant-rich foods	Protect eggs from oxidative damage	Berries, leafy greens, sweet potato, bell peppers
Healthy fats (omega-3s)	Support hormone production & cell membrane health	Salmon, sardines, walnuts, flaxseed, avocado
Whole grains	Regulate blood sugar & hormone balance	Quinoa, brown rice, oats, farro
Plant-based proteins	Reduce inflammation	Lentils, black beans, chickpeas, edamame
Folate-rich foods	Critical for cell division & DNA health	Dark leafy greens, lentils, asparagus, fortified grains
Full-fat dairy (moderate)	Linked to improved ovulatory function	Whole milk yogurt, cheese (in moderation)

Key Supplements (Discuss with Your Doctor First)

- **Prenatal vitamin with folate (methylfolate):** Start 3 months before retrieval. Look for methylfolate rather than folic acid if you have the MTHFR gene variant, which is more common in women of color.
- **CoQ10 / Ubiquinol (200–600mg/day):** A powerful antioxidant that supports mitochondrial function within eggs and may improve egg quality, especially over 30.
- **Vitamin D (1,000–2,000 IU/day):** Vitamin D deficiency is disproportionately common in women of color due to melanin's effect on UV absorption. Deficiency is linked to lower IVF success rates.
- **Omega-3 fatty acids (EPA/DHA):** Support egg membrane health and reduce inflammation.
- **Iron:** Low iron is linked to ovulatory dysfunction. Have your levels tested.

Always inform your fertility clinic of every supplement and medication you take before starting stimulation. Some supplements should be paused during stimulation or after retrieval.

Lifestyle Recommendations

Lifestyle Factor	Recommendation
Exercise	Moderate movement (walking, yoga, swimming) is encouraged. Avoid high-intensity workouts during stim
Sleep	Aim for 7–9 hours per night. Sleep deprivation disrupts reproductive hormones.
Alcohol	Minimize or eliminate. Even moderate alcohol can impact egg quality and hormone levels.
Caffeine	Limit to one cup per day. Avoid decaf (may contain residual chemicals from processing).
Smoking / Vaping	Stop as soon as possible. Smoking accelerates egg quality decline significantly.
Stress	Chronic stress raises cortisol, which disrupts hormonal balance. Prioritize mental health support.
Environmental toxins	Reduce exposure to BPA (plastics), phthalates, and pesticides. Choose clean beauty and household prod
Body weight	Both underweight and overweight status can affect egg quality and hormonal response. Work with a prov

Section 5

Finding the Right Provider

Choosing a fertility specialist is one of the most important decisions in your journey. You deserve a provider who listens, explains, and respects your full identity and background. Here is what to look for and what questions to ask.

What to Look for in a Fertility Clinic

- **Board certification:** Your doctor should be board-certified in Reproductive Endocrinology & Infertility (REI).
- **SART membership:** Clinics that report to SART are held to standardized quality and reporting benchmarks.
- **Transparent success rates:** Ask to see their CDC/SART success rate data, broken down by age group.
- **Inclusive culture:** Look for clinics that explicitly mention DEI commitments, diversity in staff, or specific programs for BIPOC patients.
- **Clear financial guidance:** A good clinic will walk you through all costs upfront and help you explore insurance or financial assistance options.
- **Patient advocacy support:** Ask if they have a patient navigator or coordinator who can support you through the process.

Questions to Ask Your Fertility Doctor

Questions About Your Fertility	Questions About the Clinic & Process
What does my AMH level indicate about my ovarian reserve?	What is your clinic's success rate for women my age?
How many eggs should I aim to freeze based on my results?	Do you have providers who specialize in caring for Black or Latina patients?
Are there any conditions (fibroids, PCOS, endometriosis) that could affect success?	What are the costs involved, including medication, monitoring, and storage?
What is your protocol for stimulation, and can it be adjusted to my needs?	Do you accept my insurance, and what does it cover?
What is my risk of OHSS, and how will you monitor for it?	What is your cancellation policy if my cycle is not progressing well?

How to Find Black & Culturally Competent Fertility Specialists

- **Cofertility's Black Fertility Doctor Directory** — A curated, verified list of Black REI specialists across the US and Canada. Visit: cofertility.com/family-learn/black-fertility-doctors
- **Shady Grove Fertility (SGF)** — Offers the FertilityEquity™ program (developed with Morehouse School of Medicine) and has Black fertility specialists on staff. Website: shadygrovefertility.com
- **Laurel Fertility Care** — San Francisco clinic led by Dr. Collin Smikle, with an explicit mission of inclusive excellence for Black and LGBTQ+ patients. Website: laurelfertility.com
- **NYU Langone Fertility Center** — Committed to culturally competent, data-driven care for all patients, with dedicated BIPOC resources. Website: fertilitynyu.org

- **RESOLVE's Clinic Finder** — The National Infertility Association's searchable database of fertility clinics.
Website: resolve.org/find-a-doctor/

Section 6

Financial Resources & Assistance

Cost is one of the largest barriers to egg freezing for women of color. A single cycle can cost \$10,000–\$15,000 in fees, plus \$5,000–\$7,000 or more in medications, and ongoing storage costs. Here are key resources and strategies to help make care more accessible.

Resource	Description	Website
Tinina Q. Cade Foundation	Grants up to \$10,000 for infertility treatment and adoption, with a focus on families of color.	tininacade.org
Baby Quest Foundation	Grants for IVF, egg freezing, and other fertility treatments for those who cannot afford care.	babyquest.org
RESOLVE Financial Resources	Comprehensive guide to loans, grants, shared-risk programs, and insurance navigation.	resolvefertility.org/financial-resources/
Compassionate Care Program	EMD Serono pharmaceutical manufacturer's medication discount program for qualified patients.	emdserono.com/compassionate-care
LendingClub / CapexMD	Specialized fertility financing loans with flexible repayment terms.	lendingclub.com/capexmd
Insurance Advocacy	As of 2024, 16 states have fertility insurance mandates. Ask your HR department or insurer about your specific coverage.	askyourhr.com
Employer Benefits	Ask your employer about fertility benefits — many large employers now cover egg freezing as part of health insurance.	askyourhr.com

Pro Tip: Ask clinics about 'shared-risk' or 'refund' programs — these guarantee a refund if no live birth occurs after a set number of IVF cycles using your frozen eggs. They require a larger upfront fee but offer financial protection. Not all clinics offer this, so ask directly.

Section 7

Support Organizations & Community Resources

You do not have to navigate this alone. These organizations specifically serve women of color and BIPOC communities on their fertility journeys — offering community, education, advocacy, and financial support.

Fertility for Colored Girls

fertilityforcoloredgirls.org

Founded in 2013 by Rev. Dr. Stacey Edwards-Dunn, this national organization provides education, advocacy, support groups, and community for Black women and couples experiencing infertility, miscarriage, and loss. Strong online community and local chapters.

The Broken Brown Egg

thebrokenbrownegg.org

A support platform created by and for people in the Black community experiencing infertility. Offers community forums, real stories, and resources to reduce isolation and stigma.

The Resilient Sisterhood Project

resilientsisters.org

Focused on educating and empowering women of African descent about reproductive diseases — including fibroids, endometriosis, and PCOS — that disproportionately affect them and are often undertreated or misdiagnosed.

Sister Girl Foundation, Inc.

sistergirlconnect.com

Founded to bring awareness to endometriosis and gynecological illnesses in the Black community, with a focus on locating endometriosis specialists and securing financial resources for treatment.

Sisters in Loss

sistersinloss.com

Holds space for Black women experiencing pregnancy loss and infertility. Provides educational resources, doula services, birth and bereavement support, and grief counseling.

Tinina Q. Cade Foundation

cadefoundation.org

A non-profit providing education and financial grants for infertility treatment and adoption, with a particular focus on helping Black families overcome the financial barriers to parenthood.

Therapy for Black Girls Directory

therapyforblackgirls.com

While not fertility-specific, this directory helps Black women find therapists who specialize in fertility grief, perinatal mental health, and reproductive trauma — an important part of the journey.

RESOLVE: The National Infertility Association

resolve.org

The leading national advocacy and support organization for all people experiencing infertility. Offers a clinic finder, support groups, insurance navigation guides, and legislative advocacy for fertility coverage mandates.

Mental Health Reminder: The fertility journey can be emotionally exhausting. Grief, anxiety, and identity stress are all normal responses. Perinatal mental health support is not a luxury — it is a part of comprehensive fertility care. The Perinatal Mental Health Alliance for People of Color bridges mental health access gaps specifically for birthing persons of color. Website: pmhapoc.org

Section 8

Master Checklist: Your Egg Freezing Journey

Use this checklist as a guide to stay organized and empowered throughout your fertility journey. Check off each item as you complete it. Every journey is different — not every step will apply to you.

■ Schedule a preconception check-up with your OB-GYN or primary care provider
■ Get baseline bloodwork: AMH, FSH, Vitamin D, iron, thyroid panel, CBC
■ Start a high-quality prenatal vitamin with methylfolate
■ Begin CoQ10/Ubiquinol supplementation (discuss dose with your doctor)
■ Begin a Mediterranean-style eating pattern: more vegetables, whole grains, healthy fats
■ Reduce alcohol intake; work toward elimination
■ If you smoke or vape, begin a cessation plan now
■ Limit caffeine to 1 cup per day
■ Start moderate exercise (walking, yoga, swimming) 3–5x per week
■ Prioritize 7–9 hours of sleep per night
■ Address chronic stress: therapy, mindfulness, support groups
■ Reduce exposure to BPA, phthalates, and synthetic fragrances where possible
■ Get Vitamin D tested; supplement if deficient (common in women of color)
■ Research SART-member clinics and their success rates by age group
■ Look for providers with explicit DEI commitments or BIPOC-focused care programs
■ Search Cofertility's Black Fertility Doctor Directory for Black REI specialists
■ Schedule consultations with at least 2 clinics before choosing
■ Ask each clinic about their stimulation protocols for patients with your profile
■ Request a complete breakdown of all costs (retrieval, medications, monitoring, storage)
■ Confirm your insurance coverage and which services are included

- Ask about financial assistance, grants, or financing programs
- Assess clinic communication: Are they responsive? Do they explain things clearly?
- Trust your gut — you should feel heard and respected at every appointment

- Review your AMH, FSH, and antral follicle count with your doctor
- Discuss any diagnoses: PCOS, fibroids, endometriosis, or blocked tubes
- Understand your predicted egg yield and what number to aim for
- Clarify which stimulation protocol your doctor recommends and why
- Ask how your OHSS risk will be monitored and managed
- Get a full medication list and injection training before your cycle begins
- Understand the monitoring schedule (blood draws + ultrasounds every 2–3 days)
- Confirm your clinic's cancellation and cycle freeze policy
- Set up a point of contact for questions during stimulation

- Take all injections at consistent times as instructed
- Attend every monitoring appointment (do not skip)
- Track any symptoms: severe bloating, shortness of breath, extreme pain — report immediately
- Avoid high-intensity exercise during stimulation
- Stay hydrated; drink electrolyte-rich fluids
- Continue taking prenatal vitamins unless advised otherwise
- Arrange for someone to drive you to and from your retrieval
- Prepare your home: heating pad, comfortable clothing, easy-to-digest foods
- Avoid alcohol and NSAIDs (like ibuprofen) unless cleared by your doctor
- Notify your workplace that you may need flexibility during monitoring days

- Rest for the remainder of retrieval day; arrange for support at home
- Confirm with your embryologist how many mature eggs were frozen
- Understand your storage terms, fees, and renewal timeline
- Keep your storage facility contact info and account details in a safe place
- Update your contact information annually with your storage facility
- Decide how long you plan to store eggs and set calendar reminders
- Connect with a community: Fertility for Colored Girls, The Broken Brown Egg, RESOLVE
- Consider counseling if you experience grief, anxiety, or emotional difficulty
- Follow up with your REI if you have any ongoing health concerns post-retrieval
- Celebrate yourself — this is a brave and powerful step

You are your own best advocate. No matter what the system has looked like for women who came before you, you deserve complete information, compassionate care, and a provider who sees and honors all of who you are. This guide is a starting point — your story is your own.

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This guide was compiled for educational purposes only. It is not a substitute for personalized medical advice from a licensed reproductive endocrinologist. Medical information is subject to change — always verify with your provider and consult current clinical guidelines.